

Ch 1: Protocol Overview

This advanced bulking cycle combines anabolic steroids with growth hormone-releasing peptides for maximum lean mass gain. Target: 8–12 kg lean mass in 16 weeks for experienced users who have already explored testosterone-only and basic compound cycles.

Parameter	Details
	16 weeks active + 2 weeks clearance + 4 weeks PCT
Compounds	Test-E 500mg/wk + Deca 300–400mg/wk + DBol kickstart (weeks 1–4) + BPC-157 + TB-500
	8–12 kg lean mass (with caloric surplus 500–700 kcal + 2g/kg protein)
Caloric Surplus	+500–700 kcal above TDEE on training days; +200–300 on rest days
	PPL × 2/week (Push/Pull/Legs twice per week — 6 sessions/week)
PCT Start	2 weeks after last Test-E injection (long ester clearance)

Ch 2: 16-Week Compound Schedule

Dianabol (DBol) is used as a kickstart — it provides rapid mass and strength gains while Test-E levels build up over weeks 1–4. DBol is discontinued at week 4 to protect the liver. The remainder of the cycle is injectable-only for superior safety and consistency.

Week	Test-E mg/wk	Deca mg/wk	DBol mg/day	Peptides	Anastrozole	Cabergoline
	500	–	30	BPC-157 250mcg 2×/day	0.5mg E3D	–
3–4	500	300	30	BPC-157 + TB-500 2mg/wk	0.5mg E3D	0.25mg 2×/wk
	500	300	–	BPC-157 250mcg 2×/day	0.5mg E3D	0.25mg 2×/wk
9–12	600	400	–	TB-500 2mg/wk	0.5mg E3D	0.25mg 2×/wk
	500	300	–	BPC-157 250mcg 2×/day	0.5mg E3D	0.25mg 2×/wk
15–16	250 / 0	150 / 0	–	–	0.25mg E3D	Taper off

Ch 3: Peptide Protocols

Peptides enhance the cycle by stimulating natural GH pulses (GHRP/GHRH synergy), accelerating tissue repair (BPC-157), and improving recovery speed (TB-500). They work through different mechanisms than anabolic steroids and provide additive benefits.

Peptide	Dose	Frequenc y	Route	Primary Benefit	Cost/Month
	250 mcg	2× daily	Subcutaneous near target tissue	Tendon/ligament protection, GI health	■2000–3500
TB-500	2 mg	Weekly	Subcutaneous any site	Muscle repair acceleration, angiogenesis	■1500–2500
	200 mcg	2× daily	Subcutaneous	GH pulse stimulation, sleep improvement	■1500–2500
CJC-1295 no DAC (optional)	100 mcg	2× daily (with Ipa)	Subcutaneous	Synergistic GH amplification	■1000–2000

Ch 4: Mass Building Nutrition

Without a caloric surplus and adequate protein, no amount of anabolic compounds will produce maximum muscle growth. The following meal plan targets 4,000–4,500 kcal/day for an 80–90 kg male.

Meal	Food	kcal	Protein
	6 eggs + 2 roti + banana + 500mL full-fat milk	880	62g
Mid-morning 10 AM	Peanut butter oats (1 cup oats + 2 tbsp PB) + whey shake	640	45g
	300g chicken/paneer curry + 2 cups rice + dal + curd	850	68g
Pre-workout 4 PM	2 roti + peanut butter + banana + creatine 5g	440	18g
	Whey shake + 50g soya chunks sabzi + 1 cup rice	540	52g
Dinner 9 PM	200g paneer + 2 roti + rajma + sabzi	640	44g
	1 cup full-fat curd + casein shake 30g	300	34g
TOTAL		4,290 kcal	323g

Ch 5: On-Cycle Support Stack

Mandatory On-Cycle Support Supplements

- Liver: TUDCA 500mg/day + NAC 600mg/day (especially weeks 1–4 with DBol)
- Blood pressure: Fish oil 4g/day + CoQ10 200mg/day + Hawthorne berry 500mg
- Taurine 5g/day (combats DBol-induced lower back pumps)
- Estrogen: Anastrozole 0.5mg E3D — adjust based on E2 blood test (target 20–35 pg/mL)
- Prolactin (from Deca): Cabergoline 0.25mg 2×/week — essential with Nandrolone
- Joints: Collagen 15g/day + Glucosamine 1500mg + BPC-157 (covered in peptide protocol)
- Sleep/recovery: Ashwagandha 600mg KSM-66 + Zinc 30mg + Magnesium glycinate 400mg

Cabergoline (Caber) — Why It's Non-Negotiable with Deca

Nandrolone (Deca) elevates prolactin via its progestogenic activity. Elevated prolactin causes: sexual dysfunction, gynecomastia that does not respond to Nolvadex (prolactin-induced, not E2-induced), emotional blunting, and reduced libido. Cabergoline 0.25mg twice per week prevents this effectively.

Ch 6: Training & PCT

A PPL × 2 split provides 12 weekly sets per muscle group — well within the 10–20 set Maximum Adaptive Volume range. High frequency combined with high protein and anabolic support produces superior hypertrophy vs lower frequency approaches.

Day	Session	Priority Exercises	Volume
	Push A (Chest dominant)	Bench press 5×5, Incline DB 4×8, Cable fly 3×12, OHP 4×6, Skull crusher 3×10	Heavy compound focus
Tuesday	Pull A (Back dominant)	Deadlift 5×5, Weighted pull-up 4×6, Barbell row 4×6, Face pull 3×15, Curl 3×10	Posterior chain priority
	Legs A (Quad dominant)	Squat 5×5, Leg press 4×10, Hack squat 3×10, Leg curl 3×12, Calf raise 5×15	Squat depth + full ROM
Thursday	Push B (Shoulder dominant)	OHP 4×6, DB shoulder press 3×10, Lateral raise 4×15, Dips 4×10, Tricep ext 3×12	Volume accumulation
	Pull B (Arm dominant)	Weighted pull-up 4×8, Cable row 3×10, Hammer curl 4×10, Reverse curl 3×12	Bicep specialisation
Saturday	Legs B (Glute/Ham dominant)	Bulgarian split squat 4×10, Romanian DL 4×8, Glute bridge 3×15, Leg curl 4×12	Posterior chain volume
	Rest / Active recovery	Yoga, foam rolling, swimming 30 min	Parasympathetic recovery

PCT — Begins 2 Weeks After Last Test-E Injection

PCT Week	Nolvadex	Clomid	Key Supplements
	40 mg/day	100 mg/day	Ashwagandha 600mg + Zinc + Vitamin D 5000IU
Week 3–4	20 mg/day	50 mg/day	Continue + Tongkat Ali 400mg + Boron 10mg